Which treatment this guide covers This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

NCCP 00353 · VERSION 5

Osimertinib monotherapy

The usual starting dose is 80 mg by mouth once daily, every day without a planned rest week. Use the dose on your prescription. Advanced treatment continues while helping and tolerated; adjuvant treatment is planned for up to three years, with earlier stopping if cancer returns or side effects become unacceptable.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Diarrhoea and mouth soreness

☐ Low blood counts

☐ Heart rhythm changes and reduced heart function

☐ Eye inflammation

☐ Rash, dry skin and nail changes

☐ Serious lung inflammation

☐ Rare serious skin reactions and aplastic anaemia

Discuss adjuvant versus advanced intent, expected benefit and uncertainty, alternatives including not proceeding, common skin/bowel effects, serious lung/heart/skin/blood problems, fertility and pregnancy, daily tablets, monitoring and your right to reconsider.

Individual benefit, alternatives and concerns

Agreed checks and next review

Confirm the relevant EGFR test, treatment setting and prescription. Adjuvant treatment needs complete resection and the specific stage/mutation criteria. Blood count, kidney, liver and bone profile; ECG before treatment. Consider an echocardiogram or MUGA LVEF assessment for cardiac risks or symptoms; review all medicines, pregnancy, contraception and breastfeeding. Repeat blood count, kidney, liver and bone profile at the agreed reviews; extra checks may be needed for symptoms or abnormal results. NCCP specifies ECG at baseline and cycle 2, then cycle 3 onwards if clinically indicated. The team translates cycle 2 into a booked review; the tablets remain daily, without a treatment break. LVEF assessment if clinically indicated; ECG and electrolytes with cardiac/QT risks. Scans and disease checks follow the Consultant plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.